

Contract Pharmacy Prevention of Diversion

Purpose: The purpose of the Contract Pharmacy Audit is to improve contract pharmacies' compliance with the 340B program requirements. Covered entities remain responsible for the 340B drugs they purchase and dispense through a contract pharmacy. Covered entities are required to ensure ongoing compliance and the timely recognition of any 340B program compliance problems at all contract pharmacy locations, while maintaining auditable records.

The contract pharmacy self-audit tools are divided into three compliance elements:

- Contract Pharmacy Eligibility
- Contract Pharmacy Prevention of Diversion
- Contract Pharmacy Prevention of Duplicate Discounts

Covered entities are encouraged to complete this tool quarterly for each contract pharmacy service organization. Proceed through the steps as follows:

1. Identify the staff member to complete audit.
2. Identify and collect relevant data as follows:
 - a. List of eligible covered entity locations (clinics/departments/service units)
 - b. List of eligible providers
 - c. Proof of provider eligibility (contract/employment records, referral for consultation)
 - d. Patients' health care records.
 - e. Contract pharmacy 340B purchasing invoice
 - f. 340B dispensing records (including signature capture records)
 - g. NDC crosswalk (for virtual inventory)
 - h. Pharmacy accumulation report (for virtual inventory)
 - i. Pharmacy inventory report (for physical inventory)
 - j. For grantee: grant that qualifies the grantee to participate in the 340B program
3. Select audit samples
 - a. Select 10 contract pharmacy invoices, as follows: 5 invoices with the highest volume (number of lines) and 5 invoices with the highest total cost.
 - b. Randomly select 10 different drugs from the 340B purchasing invoices identified in step 3a (recommend one per invoice).
 - c. Randomly select 1 dispensation for each of the 20 different drugs identified in step 3b.
 - d. Randomly select 5 prescriptions "returned to stock" (not picked up) from contract pharmacy dispensing records.
4. Complete Tables 1 through 5 and answer the corresponding assessment questions.
5. Complete the "Summary of Results."
 - a. This section is a brief summarizing statement of the self-audit results for senior leadership and other key 340B stakeholders.
6. Review the results with the 340B Steering Committee (or other compliance oversight committee as determined by entity's compliance program or policies and procedures).
 - a. Assess whether the results are indicative of a material breach (refer to Establishing Material Breach Threshold Tool



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<https://docs.340bpvp.com/documents/public/resourcecenter/establishing-material-breach-threshold.docx>)

7. Develop a corrective action plan, if applicable.
 - a. Attach a corrective action plan that addresses the compliance issues identified in this self-audit.



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Covered Entity & Contract Pharmacy Information

1. Entity's Name	
2. Entity's 340B ID	
3. Document the name of the contract pharmacy organization(s) (not locations) being audited. (A contract pharmacy organization may be a chain or an independent pharmacy and have multiple service site addresses.)	
4. Date of the LAST self-audit.	
5. Audit sample period of THIS self- audit. (Note: First day of audit sample period should be the day after the last day of the previous audit sample.)	
6. Date of THIS self-audit.	
7. Name and title of individual completing THIS self-audit.	
8. Summary of Results: Note areas for improvement identified.	
9. Actions to be taken:	





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Table 1: Assessment Questions	Yes	No	N/A	Unsure
1) Were all prescriptions tested in Table 1 filled at a contract pharmacy location registered on the 340B OPAIS?	☐	☐	☐	☐
- Identify the contract pharmacy organization locations of this self-audit. - List location where the prescription was dispensed. - Compare the location where the prescription was dispensed to the hospital/grantee's HRSA 340B OPAIS registered contract pharmacy list. Answer "Yes" to the question only if all contract pharmacy locations are listed on the 340B OPAIS. <i>If response is "No" or "Unsure," indicate Rx # and explain:</i>				
2) For each prescription tested in Table 1, did each patient receive the prescription from an outpatient service at an eligible site/department registered on the 340B OPAIS?	☐	☐	☐	☐
- Identify the service/visit from which the prescription was written. - Match the service/visit location with an eligible clinic/department registered on the 340B OPAIS. Answer "Yes" to the question only if all answers are YES in the columns titled "Drug prescribed from 340B eligible location" and "Drug prescribed from location with a 340B ID" in Table 1. <i>If response is "No" or "Unsure," indicate Rx # and explain:</i>				
3) For each prescription tested in Table 1, did each patient receive the prescription from an eligible provider employed or contracted with the hospital/grantee?	☐	☐	☐	☐
- Identify prescriptions that originated from a hospital/grantee site registered on the HRSA 340B OPAIS. - List provider who prescribed the medication deemed 340B eligible. - Compare the prescribing provider to the eligible provider list. - Determine the eligibility of provider. Answer "Yes" to the question only if all answers are YES in the column titled Eligible Provider: "Employed/contracted" in Table 1. <i>If response is "No" or "Unsure," indicate Rx # and explain:</i>				



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4) **For any prescription tested in Table 1 that *originated from referral*: Does a referral for consultation and referral visit summary exist in the patient's health record for each prescription not written from a hospital/grantee site registered on the HRSA 340B OPAIS?**

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- Identify prescriptions that did NOT originate from a hospital/grantee site registered on the HRSA 340B OPAIS.
- Determine the hospital/grantee referring provider.
- List providers who prescribed the drug deemed 340B eligible.
- Locate a documented referral for consultation to the outside provider.
- —Locate a documented summary of the referral visit from the outside (referred) provider.

If no prescriptions arose from referral for consultations, indicate N/A. Otherwise, answer "Yes" only if all answers are YES in the column titled "Eligible Provider: Documented Referral" in Table 1.

If response is "No" or "Unsure," indicate which Rx # and explain:

5) **Was each prescription tested in Table 1 documented in the patient's health care record? (Demonstrates that the hospital or grantee maintains responsibility for the care of the patient.)**

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Answer "Yes" to the question only if all answers are YES in the column titled "Documented in the health care record" in Table 1.

If response is "No" or "Unsure," indicate which Rx # and explain:

6) **For Grantee Only: Was each prescription tested in Table 1 a result of a service consistent with the scope of grant funding?**

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Identify which services are included within the grant.

Answer "Yes" to the question only if all answers are YES in the column titled "Services included in scope of grant" in Table 1.

If response is "No" or "Unsure," indicate which Rx # and explain:

7) **For each prescription tested in Table 1, does the contract pharmacy's dispensing record accurately reflect the data on the prescription? (Demonstrates accurate tracking and accounting of 340B drugs.)**

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Compare the electronic dispensing data (date written, patient, prescriber, drug name, strength, quantity, and days' supply) with the actual prescription (paper or electronic copy).





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Table 2: Assessment Questions	Yes	No	N/A	Unsure
1) For the drugs tested in Table 2 during the defined time period, does the number of units purchased and dispensed reconcile to the number of units left in inventory?	☐	☐	☐	☐
Answer "Yes" to the question only if all answers are YES in the column titled "Reconciled" in Table 2.				
If response is "No" or "Unsure," explain:				
2) For the drugs tested in Table 2 during the defined time period, were all identified variances resolved and documented? <i>(Demonstrates that variances were not the result of diversion)</i>	☐	☐	☐	☐
Answer "Yes" to the question only if all answers are YES in the column titled "Variance Resolved" in Table 2.				
If response "No" or "Unsure," explain: <i>(Identify and discuss each outlier)</i>				
3) If a <u>virtual inventory</u> is used, is the accumulator manually adjusted when an NDC ordered is not available and the wholesaler ships a substitution?	☐	☐	☐	☐
Answer N/A if a physical inventory is used.				
- Identify actual invoices when substitutions occurred. - Verify that a manual adjustment to the accumulator was made.				
Answer "Yes" to the question only if substitutions were made and a manual adjustment to the accumulator was documented.				
If response is "No" or "Unsure," indicate Rx # and explain: <i>(Identify and discuss each outlier)</i>				
(continue on next page)				





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Table 3: Assessment Questions	Yes	No	N/A	Unsure
1) For the drugs tested in Table 3, was the dispensed quantity of medication correctly accumulated?	☐	☐	☐	☐
Answer N/A if a physical inventory is used. Answer "Yes" to the question only if all answers are YES in the column titled "Drug NDC quantity dispensed matches quantity accumulated" in Table 3. <i>If response is "No" or "Unsure," indicate Rx # and explain: (Identify and discuss each outlier)</i>				
2) For the drugs tested in Table 3, did the 11-digit NDC billed match the 11-digit NDC accumulated for each accumulation sample?	☐	☐	☐	☐
Answer N/A if a physical inventory is used. Answer "Yes" to the question only if all the answers are YES in the column titled "NDC billed matches NDC accumulated" in Table 3. <i>If response is "No" or "Unsure," indicate which Rx # and explain: (Identify and discuss each outlier)</i>				
3) For the drugs tested in Table 3, did the drug NDC and quantity ordered on the pharmacy's 340B account match the drug NDC and quantity deducted from the 340B accumulator? <i>(Quantity in accumulator are based on billing units per package [BUPP].)</i>	☐	☐	☐	☐
Answer N/A if a physical inventory is used. Answer "Yes" to the question only if all answers are YES in the column titled "Drug NDC and quantity ordered matches drug NDC and quantity deducted from 340B accumulator" in Table 3. <i>If response "No" or "Unsure," indicate Rx # and explain:</i>				
4) For the drugs tested in Table 3, did the drug NDC and quantity received match the drug NDC and quantity ordered?	☐	☐	☐	☐
Answer N/A if a physical inventory is used. Answer "Yes" to the question only if all the answers are YES in the column titled "Drug NDC and quantity received matches drug NDC and quantity ordered" in Table 3. <i>If response "No" or "Unsure," indicate Rx # and explain: (Identify and discuss each outlier)</i>				



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PHYSICAL INVENTORY				
Answer Assessment Questions	Yes	No	N/A	Unsure
1) If a <u>physical inventory</u> is used, are 340B drugs on the shelves identifiable or isolated from non-340B drugs?	☐	☐	☐	☐
Answer N/A if a virtual inventory is used. To answer this question, the reviewer will be required to visit the medication storage area of the contract pharmacy involved in the self-audit.				
<i>If response is "No" or "Unsure," explain:</i>				
2) If a <u>physical inventory</u> is used, does a system exist to validate that the drug name, strength, and quantity on the wholesaler invoice are received at the contract pharmacy?	☐	☐	☐	☐
Answer N/A if a virtual inventory is used. For each drug listed in Table 2, choose one invoice date to test. Identify and reconcile the actual wholesaler packing slip with actual invoice.				
<i>If response "No" or "Unsure," explain: (Identify and discuss each outlier)</i>				



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RETURN TO STOCK VERIFICATION

Table 4

- For each of the 5 prescriptions selected in step 3d of the instructions (page 1), verify that the accumulator or physical on-hand amount was manually adjusted to reflect the return of inventory to stock.

TABLE 4 Return to Stock Process Verification Table Time period tested: begin date _____ to end date _____ (attach data to substantiate inventory adjustment for each sample)					
(1) Prescription #	(2) Date filled	(3) Date returned to stock	(4) Manual adjustment of accumulator or physical on-hand amount?		(5) Date manual adjustment documented
			YES	NO	

Table 4: Assessment Question

1) For the drugs tested in Table 4, was the accumulator or physical on-hand amount manually adjusted for "return to stocks"?

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Identify when a prescription was returned to stock.

Verify that a manual adjustment to the accumulator or physical on-hand amount was made.

Answer "Yes" to the question only if all answers are YES in the column titled "Manual adjustment of accumulator or physical on-hand amount" in Table 4.

If response is "No" or "Unsure," explain:



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CONTRACT PHARMACY PURCHASING ACCOUNTS

Table 5

- List contract pharmacy purchasing accounts for each contract pharmacy location.

TABLE 5 Contract Pharmacy Purchasing Accounts Table Date documented			
(1) Contract Pharmacy Location	(2) 340B (Wholesaler/ Direct/Account #)	(3) WAC (Wholesaler/ Direct/Account #)	(4) GPO (Wholesaler/ Direct/Account #)

Table 5: Assessment Question

1) **DSH, PED, CAN:** For contract pharmacies listed in Table 5, are all the hospital's drugs shipped to the contract pharmacies ordered through either a 340B account or WAC account?

Yes No N/A Unsure

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- List contract pharmacy purchasing accounts for each contract pharmacy location.
- Verify that the covered entity drugs are not purchased using a GPO account at contract pharmacy.

Answer N/A if covered entity is not subject to the GPO Prohibition.

Answer "Yes" to the question only if accounts are listed in the columns titled "340B" and "WAC" in the contract pharmacy purchasing accounts.

If response is "No" or "Unsure," explain: